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Two deadlines and a database: what EU MDR now asks of Turkish medical device manufacturers

Legacy certificates run out at the end of 2027 and 2028, and EUDAMED registration has been mandatory since 28 May 2026. A revision is on the table, but it does not move these dates.

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Executive summary

1. Under the EU Medical Device Regulation, Regulation (EU) 2017/745, devices still sold on certificates issued under the old directives can stay on the EU market only until 31 December 2027 for class III and implantable class IIb devices, and 31 December 2028 for other class IIb, class IIa and certain class I devices. Regulation (EU) 2023/607 granted these extensions on strict conditions, including an application to a notified body by 26 May 2024 and a signed agreement by 26 September 2024.
2. Since 28 May 2026 four modules of the European database EUDAMED have been mandatory: actor registration, UDI and device registration, notified bodies and certificates, and market surveillance. Manufacturers must register devices and keep the data current, with transitional periods for devices already on the market.
3. The Commission proposed a targeted revision of the MDR on 16 December 2025. It could ease certificate renewal and some documentation, but it was still in the legislative process in October 2026 and does not change the 2027 and 2028 deadlines. For a Turkish manufacturer the decision is portfolio triage: which devices to certify in time, which to redesign, and which to withdraw.

What has changed

The MDR has applied since 26 May 2021. Devices certified under the former Medical Devices Directive or Active Implantable Medical Devices Directive, so-called legacy devices, were given transitional periods that Regulation (EU) 2023/607 extended in March 2023.

The extension is conditional. The manufacturer had to have a quality management system under the MDR in place by 26 May 2024, to have lodged a formal application with a notified body by the same date, and to have signed a written agreement with the notified body by 26 September 2024. The device must not undergo significant changes in design or intended purpose. Devices that missed these conditions are no longer covered by the extension.

Regulation (EU) 2024/1860 introduced a gradual roll-out of EUDAMED and, since 10 January 2025, an obligation for manufacturers to inform authorities and customers in advance when they expect to interrupt or discontinue the supply of certain devices.

Commission Decision (EU) 2025/2371, published on 27 November 2025, confirmed that four EUDAMED modules were functional. Their use became mandatory six months later, on 28 May 2026. The clinical investigation and vigilance modules follow later.

On 16 December 2025 the Commission proposed a targeted revision of the MDR and the In Vitro Diagnostic Regulation. Among other changes, it would replace the fixed five-year certificate validity with risk-based periodic reviews and allow more information to be supplied digitally. In October 2026 Parliament and Council were still examining it, with adoption unlikely before late 2027.

Türkiye aligned its Medical Device Regulation with the MDR in 2021, and devices are registered in the national Product Tracking System (ÜTS) run by the Turkish Medicines and Medical Devices Agency (TİTCK).

The decision behind the requirement

For many Turkish manufacturers the binding constraint is notified body capacity and the cost of clinical evidence, not the text of the law. Each legacy device needs a clear route: certify under the MDR before its deadline, change the design and certify a new device, or stop selling it in the EU.

That makes the MDR a portfolio decision. Devices with strong EU sales and good clinical data justify the cost. Low-volume variants may be better consolidated or withdrawn. Distributors and hospital buyers need to know the plan early, and since 2025 the law requires advance notice of interruptions of supply.

EUDAMED adds a data discipline. Actor, device and certificate data must be accurate, consistent with labels and kept current. Errors are now visible to authorities and, in part, to the public.

Five common misreadings

“The deadlines were extended, so we have time.” Only for devices that met the 2024 conditions, and only until the end of 2027 or 2028. Notified body reviews take many months.

“The revision will push the deadlines again.” The December 2025 proposal targets how the system works, not the 2027 and 2028 dates, and it will not be adopted in time to help most legacy devices.

“EUDAMED is the notified body’s job.” Manufacturers register themselves and their devices; notified bodies upload certificates.

“Our Turkish registration covers the EU.” ÜTS registration and EUDAMED registration are separate, even though the rules are aligned.

“Class I devices are unaffected.” Some class I devices, such as sterile or measuring devices and those up-classified under the MDR, are inside the transitional regime.

Questions for the leadership team

1. Which of our devices are still on the EU market under legacy certificates, and which deadline applies to each?
2. Did each of them meet the 2024 application and agreement conditions?
3. Where is each device in the notified body review, and what evidence is still missing?
4. Which devices carry enough EU revenue to justify full MDR certification?
5. Which variants could be consolidated, redesigned or withdrawn?
6. Are our actor, UDI and device data in EUDAMED complete and consistent with our labels?
7. Who notifies distributors and authorities if supply of a device will be interrupted?
8. How would the proposed revision change our plans after 2028, if adopted?

What a good first step produces

A useful first step is a portfolio triage: every device sold into the EU, its certificate basis and deadline, its revenue, its evidence gaps and a recommended route. It should leave leadership with a sequenced plan for the notified body, a list of devices to consolidate or withdraw, and an owner for EUDAMED data.

Ganrich Advisory runs this with your regulatory, clinical and commercial teams, and coordinates with notified bodies and regulatory specialists. We are not a notified body and do not prepare clinical evaluations. We share the working method in a conversation rather than in print, because it depends on the portfolio.

Start a conversation

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About this paper. This paper reflects official sources checked on 24–25 October 2026. It is general information, not legal, customs, tax or engineering advice. Rules, thresholds and dates change; confirm them against official sources before acting. It describes no client work or results.

Sources: Regulation (EU) 2017/745 on medical devices: eur-lex.europa.eu/eli/reg/2017/745/oj · Regulation (EU) 2023/607 amending the transitional provisions: eur-lex.europa.eu/eli/reg/2023/607/oj · Regulation (EU) 2024/1860 on EUDAMED roll-out and supply interruption: eur-lex.europa.eu/eli/reg/2024/1860/oj · European Commission, EUDAMED: health.ec.europa.eu/medical-devices-eudamed

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